FAX TRANSMITTAL

Sending clinic: West End Consultants
Date received: 2026-01-11
Packet: 006
Documents: 3 clinical notes plus laboratory report

Contents
---
1. Hematology - Oncology -- BCCa Excision - Cheek
2. Hospice - Palliative Care -- Hospice Visit - Colon Cancer
3. IME-QME-Work Comp etc. -- IME & Record Review - Orthopedic

Specialty: Hematology - Oncology
Sample Name: BCCa Excision - Cheek

PREOPERATIVE DIAGNOSES,1. Basal cell carcinoma, right cheek.,2. Basal cell carcinoma, left cheek.,3. Bilateral ruptured silicone gel implants.,4. Bilateral Baker grade IV capsular contracture.,5. Breast ptosis.,POSTOPERATIVE DIAGNOSES,1. Basal cell carcinoma, right cheek.,2. Basal cell carcinoma, left cheek.,3. Bilateral ruptured silicone gel implants.,4. Bilateral Baker grade IV capsular contracture.,5. Breast ptosis.,PROCEDURE,1. Excision of basal cell carcinoma, right cheek, 2.7 cm x 1.5 cm.,2. Excision of basal cell carcinoma, left cheek, 2.3 x 1.5 cm.,3. Closure complex, open wound utilizing local tissue advancement flap, right cheek.,4. Closure complex, open wound, left cheek utilizing local tissue advancement flap.,5. Bilateral explantation and removal of ruptured silicone gel implants.,6. Bilateral capsulectomies.,7. Replacement with bilateral silicone gel implants, 325 cc.,INDICATIONS FOR PROCEDURES,The patient is a 61-year-old woman who presents with a history of biopsy-proven basal cell carcinoma, right and left cheek. She had no prior history of skin cancer. She is status post bilateral cosmetic breast augmentation many years ago and the records are not available for this procedure. She has noted progressive hardening and distortion of the implant. She desires to have the implants removed, capsulectomy and replacement of implants. She would like to go slightly smaller than her current size as she has ptosis going with a smaller implant combined with capsulectomy will result in worsening of her ptosis. She may require a lift. She is not consenting to lift due to the surgical scars.,PAST MEDICAL HISTORY,Significant for deep venous thrombosis and acid reflux.,PAST SURGICAL HISTORY,Significant for appendectomy, colonoscopy and BAM.,MEDICATIONS,1. Coumadin. She stopped her Coumadin five days prior to the procedures.,2. Lipitor,3. Effexor.,4. Klonopin.,ALLERGIES,None.,REVIEW OF SYSTEMS,Negative for dyspnea on exertion, palpitations, chest pain, and phlebitis.,PHYSICAL EXAMINATION,VITAL SIGNS: Height 5'8", weight 155 pounds.,FACE: Examination of the face demonstrates basal cell carcinoma, right and left cheek. No lesions are noted in the regional lymph node base and no mass is appreciated.,BREAST: Examination of the breast demonstrates bilateral grade IV capsular contracture. She has asymmetry in distortion of the breast. No masses are appreciated in the breast or the axilla. The implants appear to be subglandular.,CHEST: Clear to auscultation and percussion.,CARDIOVASCULAR: Regular rate and rhythm.,EXTREMITIES: Show full range of motion. No clubbing, cyanosis or edema.,SKIN: Significant environmental actinic skin damage.,I recommended excision of basal cell cancers with frozen section control of the margin, closure will require local tissue flaps. I recommended exchange of the implants with reaugmentation. No final size is guaranteed or implied. We will decrease the size of the implants based on the intraoperative findings as the size is not known. Several options are available. Sizer implants will be placed to best estimate postoperative size. Ptosis will be worse following capsulectomy and

Specialty: Hematology - Oncology
Sample Name: BCCa Excision - Cheek

going with a smaller implant. She may require a lift in the future. We have obtained preoperative clearance from the patient's cardiologist, Dr. K. The patient has been taken off Coumadin for five days and will be placed back on Coumadin the day after the surgery. The risk of deep venous thrombosis is discussed. Other risk including bleeding, infection, allergic reaction, pain, scarring, hypertrophic scarring and poor cosmetic resolve, worsening of ptosis, exposure, extrusion, the rupture of the implants, numbness of the nipple-areolar complex, hematoma, need for additional surgery, recurrent capsular contracture and recurrence of the skin cancer was all discussed, which she understands and informed consent is obtained.,PROCEDURE IN DETAIL,After appropriate informed consent was obtained, the patient was placed in the preoperative holding area with **** input. She was then taken to the major operating room with ABCD Surgery Center, placed in a supine position. Intravenous antibiotics were given. TED hose and SCDs were placed. After the induction of adequate general endotracheal anesthesia, she was prepped and draped in the usual sterile fashion. Sites for excision and skin cancers were carefully marked with 5 mm margin. These were injected with 1% lidocaine with epinephrine.,After allowing adequate time for basal constriction hemostasis, excision was performed, full thickness of the skin. They were tagged at the 12 o'clock position and sent for frozen section. Hemostasis was achieved using electrocautery. Once margins were determined to be free of involvement, local tissue flaps were designed for advancement. Undermining was performed. Hemostasis was achieved using electrocautery. Closure was performed under moderate tension with interrupted 5-0 Vicryl. Skin was closed under loop magnification paying meticulous attention and cosmetic details with 6-0 Prolene. Attention was then turned to the breast, clothes were changed, gloves were changed, incision was planned and the previous inframammary incision beginning on the right incision was made. Dissection was carried down to the capsule. It was extremely calcified. Dissection of the anterior surface of the capsule was performed. The implant was subglandular, the capsule was entered, implant was noted to be grossly intact; however, there was free silicone. Implant was removed and noted to be ruptured. No marking as to the size of the implant was found.,Capsulectomy was performed leaving a small portion in the axilla in the inframammary fold. Pocket was modified to medialize the implant by placing 2-0 Prolene laterally in mattress sutures to restrict the pocket. In identical fashion, capsulectomy was performed on the left. Implant was noted to be grossly ruptured. No marking was found for the size of the implant. The entire content was weighed and found to be 350 grams. Right side was weighed and noted to be 338 grams, although some silicone was lost in the transfer and most likely was identical 350 grams. The implants appeared to be double lumen with the saline portion deflated. Completion of the capsulectomy was performed on the left.

Specialty: Hospice - Palliative Care
Sample Name: Hospice Visit - Colon Cancer

HISTORY OF PRESENT ILLNESS:, History as provided primarily by the patient's daughter, as well as the referring physician revealed an approximately two-year history of colon cancer initially diagnosed when the patient presented with a swelling in his groin. Approximately one month ago, he presented with abdominal pain and presented to the hospital with transverse colon obstruction. He had a diverting colostomy performed approximately one month ago. Approximately two weeks ago, he was admitted to hospital with infection of this with chronically swollen lower extremities and is home now for approximately one week. He was deemed not to be a candidate for chemotherapy or radiation on the basis of extensive disease, as well as a longstanding history of cirrhosis with esophageal varices. Additional history includes an enlarged heart and chronic lower extremity edema associated with trauma from his time as an army infantryman in Korea many decades ago. The patient is alert and lives alone, although the daughter Ruby is in from out of town for several weeks to care for him. He denies any particular problems with the exception of itch and a site of leakage around his ostomy site. His appetite is notably improved since discharge from hospital and both he and his daughter believe he has gained a few pounds of weight. His stooling is regular. There is no fever. Of greatest concern to his daughter is a possibility that his colostomy might be reversible and at the recommendation of some of the physicians at the referring hospital, he was to have had a PET scan to assess whether the ostomy is reversible for various reasons, primarily insurance. The PET scan has not been done and the family is quite concerned about a potential surgical intervention. The patient denies anxiety or depression and there is no history of same. He was married for over 50 years and now widowed for nearly 10. He is a stepfather to five children and he has seven of his own, all in all raising 12 children. His daughters, Ruby and Camilla are most involved with the patient's care. The patient is retired, worked in supermarkets for many years when he is very proud of his time as an infantryman in Korea. He did sustain an injury to the right eye during his service. He has lots of children, grandchildren, and great-grandchildren and seems to derive great pleasure from them. He denies spiritual or religious distress. As to advanced directives, the patient appears not to have any significant advanced directives, written or oral. Family apparently is working on a "plan.",MEDICATIONS:, Medications include Toprol 12.5 mg twice daily, Lasix 20 mg daily, ranitidine 75 mg daily, potassium 20 mEq daily, Benadryl 25-50 mg at the hour of sleep as needed, not typically taken and a prescription in the house for Keflex 500 mg q.i.d. for red legs has not yet begun.,PHYSICAL EXAMINATION:, Examination reveals am alert pale, but thin gentleman with evident wasting. He is seated and walks without assistance. His blood pressure is 135/75. Pulse is 80. Respiratory rate is 14. He is afebrile. Head is without icterus. Pupils are equal and round. He has a red dry tongue without leaking mucosal

Specialty: Hospice - Palliative Care
Sample Name: Hospice Visit - Colon Cancer

lesions. There is no jugular venous distention. The chest has increased AP diameter with good air entry bilaterally. There is a systolic ejection murmur heard over the entire precordium. The rate is regular. The abdomen, he has a right-sided colostomy with a prolapsed bowel. There is an area of approximately 5 cm x 8 cm of erythema adjacent to the ostomy. The skin is intact. The bowel sounds are active. There is no tenderness in the abdomen and no palpable masses. Extremities show chronic lymphedema. The right lower extremity has an 8 cm x 10 cm patch over the anterior shin that is darkened, but not red and not warm. The distal pulses are intact. Rectal exam shows no external nor internal hemorrhoids. No mass is felt and no blood on the gloved finger. Neurologically, he is alert. He is oriented times three. His speech is clear. His mood appears to be good. His short-term memory is intact. There is no focal neurological deficit.,ASSESSMENT:, A 77-year-old gentleman with apparent widespread intra-abdominal spread of rectal cancer. Status post bowel obstruction. Comorbid cirrhosis with esophageal varices. No history of bleeding. The patient has had significant clinical and functional decline and I expect that his prognosis would be measured in weeks to months. The patient lives alone and is currently being very well cared for his daughter from out of town. She will be leaving in a few weeks. There is another in-town daughter; however, she works and has a large family.,PLAN:, I will communicate with the patient's referring physician to ascertain what clinical course and data is available. A moisture barrier will be applied to his peri-ostomy wound today and we would reassess within 24 hours. It would be appropriate for a family meeting to be scheduled to review the family and the patient's understanding of his clinical condition and to begin to address an appropriate plan of care for the patient's inevitable decline.,I spoke with Dr. Abc, who informed me that family has in their possession a disc with the CAT scan results. We will try to ask radiologic colleagues when we obtain the disc to give us a formal reading so that we might better understand the patient's clinical condition and better inform family of his clinical status.

Specialty: IME-QME-Work Comp etc.
Sample Name: IME & Record Review - Orthopedic

INTRODUCTION: , The opinions expressed in this report are those of the physician. The opinions do not reflect the opinions of Evergreen Medical Panel, Inc. The claimant was informed that this examination was at the request of the Washington State Department of Labor and Industries (L&I). The claimant was also informed that a written report would be sent to L&I, as requested in the assignment letter from the claims manager. The claimant was also informed that the examination was for evaluative purposes only, intended to address specific injuries or conditions as outlined by L&I, and was not intended as a general medical examination.,CHIEF COMPLAINTS: , This 51-year-old married male presents complaining of some right periscapular discomfort, some occasional neck stiffness, and some intermittent discomfort in his low back relative to an industrial fall that occurred on November 20, 2008.,HISTORY OF INDUSTRIAL INJURY:, This patient was injured on November 20, 2008. He works at the Purdy Correctional Facility and an inmate had broken some overhead sprinklers, the floor was thus covered with water and the patient slipped landing on the back of his head, then on his back. The patient said he primarily landed on the left side. After the accident he states that he was generally stun and someone at the institute advised him to be evaluated. He went to a Gig Harbor urgent care facility and they sent him on to Tacoma General Hospital. At the Tacoma General, he indicates that a whiplash and a concussion were diagnosed and it was advised that he have a CT scan. The patient describes that he had a brain CT and a dark spot was found. It was recommended that he have a followup MRI and this was done locally and showed a recurrent acoustic neuroma. Before, when the patient initially had developed an acoustic neuroma, the chiropractor had seen the patient and suggested that he have a scan and this was how his original acoustic neuroma was diagnosed back in October 2005. The patient has been receiving adjustments by the chiropractor since and he also has had a few massage treatments. Overall his spine complaints have improved substantially.,After the fall, he also saw at Prompt Care in the general Bremerton area, XYZ, an Osteopathic Physician and she examined him and released him full duty and also got an orthopedic consult from XYZ. She ordered an MRI of his neck. Cervically this showed that he had a mild disc bulge at C4-C5, but this actually was the same test that diagnosed a recurrent acoustic neuroma and the patient now is just recovering from neurosurgical treatment for this recurrent acoustic neuroma and some radiation is planned.,Since 2002 the patient has been seeing the chiropractor, XYZ for general aches and pain and this has included some treatments on his back and neck.,CURRENT SYMPTOMS: ,The patient describes his current pain as being intermittent.,PAST MEDICAL HISTORY:,Illnesses: The patient had a diagnosis in 2005 of an acoustic neuroma. It was benign, but treated neurosurgically. In February 2004 and again in August 2009 he has had additional treatments for recurrence and he currently has some skull

markers in place because radiation is planned as a followup, although the tumor was still indicated to be benign.,Operations: He has a history of an old mastoidectomy. He has a past history of removal of an acoustic neuroma in 2005 as noted.,Medications: The patient takes occasional Tylenol and occasional Aleve.,Substance Use:,Tobacco: He does not smoke cigarettes.,Alcohol: He drinks about five beers a week.,FAMILY HISTORY:, His father died of mesothelioma and his mother died of Lou Gehrig's disease.,SOCIOECONOMIC HISTORY:,Marital Status and Dependents: The patient has been married three times; longest marriage is of two years duration. He has two children. These dependents are ages 15 and twins and are his wife's dependents.,Education: The patient has bachelor's degree.,Military History: He served six years in the army and received an honorable discharge.,Work History: He has worked at Purdy Correctional Institute in Gig Harbor for 19 years.,CHART REVIEW: , Review of the chart indicates a date of injury of November 20, 2008. He was seen at Tacoma General Hospital with a diagnosis of head contusion and cervical strain. He had a CT of his head done because of a fall with possible loss of consciousness, which showed a left cerebellar hypodensity and further evaluation was recommended. He has a history of an old mastoidectomy. He was then seen on November 24, 2008 by XYZ at Prompt Care on November 24, 2008. There is no clearcut history that he had lost consciousness. He has a past history of removal of an acoustic neuroma in 2005 as noted. A diagnosis of concussion and cervical strain status post fall was made along with an underlying history of abnormal CT and previous resection of an acoustic neuroma. Some symptoms of loss of balance and confusion were noted. She recommended additional testing and neurologic evaluation.,The notes from the treating chiropractor begin on November 24, 2008. Adjustments are given to the cervical, thoracic, and lumbar spine.,He was seen back by XYZ on December 9, 2008 and he had been released to full duties. It was recognized the new MRI suggested recurrence of the acoustic neuroma and he was advised to seek further care in this regard. There were some concerns of his feeling of being wobbly since the fall which might be related to the recurrent neuroma. He continued to have chiropractic adjustments. He was seen back at Prompt Care on January 8, 2009. Dr. X indicated that she thought most of his symptoms were related to the tumor, but that the cervical and thoracic stiffness were from the fall.,A followup note by his chiropractor on January 26, 2009 indicates that cervical x-rays have been taken and that continued chiropractic adjustments along with manual traction would be carried out.,On April 13, 2009, he was seen again at Prompt Care for his cervical and thoracic strain. He was indicated to be improving and there was suggestion that he has some physical therapy and an orthopedic consult was felt appropriate. Therapy was not carried out and obviously was then involved with the treatment of his recurrent neuroma.,On April 17, 2009, he was seen by Dr. X, another chiropractor for consultation and further

Specialty: IME-QME-Work Comp etc.
Sample Name: IME & Record Review - Orthopedic

chiropractic treatments were recommended based on cervical and thoracolumbar subluxation complexes and strain.,A repeat consult was carried out on April 29, 2009 by XYZ. He felt that this was hyperextension cervical injury. It might take a period of time to recover. He mentioned that the patient might have a slight ulnar neuropathy. He felt the patient was capable of full duty and the patient was at that time having ongoing treatment for his neuroma.,This concludes the chart review.,PHYSICAL EXAMINATION: , The patient is 6 feet in height, weighs 255 pounds.,Orthopedic Examination: He can walk with a normal gait, but he has, as indicated, a positive Romberg test and he himself has noticed that if he closes his eyes he loses his balance. Overall the patient is a seemingly good historian. There is a visible 3 cm scar at the left base of the neck near the hairline and there are multiple areas where his head has been shaved both anteriorly and posteriorly. These are secondary to drawing for the skull markers. There is a scar behind the patient's left ear from the original treatment of the acoustic neuroma. This was well healed. The patient can perform a toe-heel gait without difficulty. One visibly can see that he has some facial asymmetry and he indicates that the acoustic neuroma has caused some numbness in the left side of his face and also some asymmetry that is now recovering. The patient states he now thinks his recovery is going to get disregarded and that the facial asymmetry and numbness developed from the first surgery he had. The patient has a full range of motion in both of his shoulders. The patient has a full range of motion in his lumbar spine to include 90 degrees of forward bend, lateral bending of 30 degrees in either direction and extension of 10 degrees. There is full range of motion in the patient's cervical spine to include flexion of 50 degrees at which time he can touch his chin on his chest. He extends 40 degrees, laterally bends 30 degrees, and rotates to 80 degrees in either direction. There is slight tenderness on palpating over the right cervical musculature. There is no evidence of any cervical or lumbar muscle spasms. Reflexes in the upper extremities include 1+ biceps and triceps and 1+ brachioradialis. Knee jerks are 2+ and ankle jerks are 1+. Tinel's test was tested at the elbow, it is negative bilaterally with percussion; however, he has slight tingling bilaterally. The patient's grip tested with a Jamar dynamometer increases from 70 to 80 pounds bilaterally. Sensory testing of lower extremities reveal that the patient has slightly decreased sensation to sharp stimulus in his dorsal aspect of the right first toe and a lesser extent to the left. Testing of muscle strength in the upper and lower extremities is normal. The patient upper arms measured four fingerbreadths above the flexion crease of the elbow measure 35 cm bilaterally. The forearms measured four fingerbreadths below the flexion crease of the elbow measure 30 cm bilaterally. The thighs measured four fingerbreadths above the superior pole of the patella measure 48 cm and the lower legs measured four fingerbreadths below the tibial tubercles measure 41 cm. Pressure on the

vertex of the head does not bother the patient. Axial loading is negative. As already indicated straight leg raising is entirely negative both sitting and lying for any radiculitis.,DIAGNOSTIC STUDIES: , X-rays the patient brings with him taken by his treating chiropractor dated 11/24/08 showed that there appears to be a little bit of narrowing of the L4-5 disc space. The hip joints are normal. Views of his thoracic spine are normal. Cervical x-rays are in the file. These are of intermittent quality, but the views do show a very slight degree of anterior spurring at the C4-5 with possible slight narrowing of the disc. There is a view of the right shoulder that is unremarkable.,CONCLUSIONS:, The accepted condition under the claim is a sprain of the neck, thoracic, and lumbar.,DIAGNOSES: , Diagnosis based on today's examination is a sprain of the cervical spine and lumbar spine superimposed upon some early degenerative changes.,Additional diagnosis is one of recurrent acoustic neuroma, presumably benign with upcoming additional treatment of radiation plan. The patient also has a significant degree overweight for his height and it will be improved as he himself recognizes by some weight loss and exercise.,DISCUSSION: , He is fixed and stable at this time and his industrial case can be closed relative to his industrial injury of November 20, 2008. Further chiropractic treatments would be entirely palliative and serve no additional medical purpose due to the fact that he has very minimal symptoms and a basis for these symptoms based on mild or early degenerative changes in both cervical and lumbar spine. He is category I relative to the cervical spine under 296-20-240 and category I to the lumbosacral spine under WAC 296-20-270. His industrial case should be closed and there is, as indicated, no basis for any disability award.

LABORATORY REPORT

Packet: 006

Analyte	Value	Units	Flag
Glucose	142	mg/dL	High
Hemoglobin	11.8	g/dL	Low
WBC	12.4	10*3/uL	High
Sodium	136	mmol/L	
Potassium	4.2	mmol/L	
Creatinine	1.4	mg/dL	High
BUN	24	mg/dL	High
Platelets	228	10*3/uL	
Hemoglobin A1c	7.6	%	High
TSH	2.1	mIU/L	